Suicide & Violence Risk Factors Ranked by Relative Risk

Companion to the Safety Risk Formulation teaching app · https://lordkelv.github.io/safety-risk-formulation/ · Generated November 8, 2026

How to read this. Each domain lists every factor in the app, ranked by its point estimate from highest to lowest, with risk factors (estimate ≥ 1) and protective factors (< 1) in separate tables.

- **Measures are not interchangeable.** Odds ratios (OR), relative risks (RR), hazard ratios (HR), standardized mortality ratios (SMR) and rate ratios come from different designs. The ranking is by point estimate only and is a rough ordering, not a strict comparison.
- **Comparator matters most.** Filled dots compare people with the factor to the general population. Hollow dots compare within an already-selected group (e.g., other people with psychosis, other abused women), so those figures are not population multipliers.
- **Outcomes differ:** suicide death vs attempt; violence perpetration vs homicide victimization of household members. Check the Outcome column before comparing rows.
- **Do not multiply rows.** Factors cluster in the same people, so products overstate risk. Risk factors predict suicide only slightly better than chance (Franklin 2017); "high-risk" categories miss ~44% of suicides (Large 2016).
- **Tags:** ✓✓ same figure found independently by both research sources (Gemini 4 Argon, GPT-6.1 Sol); ≠ sources reported different figures (explanation in notes); derived = calculated from two published estimates; ⚑ red flag = needs clinical action regardless of any number.

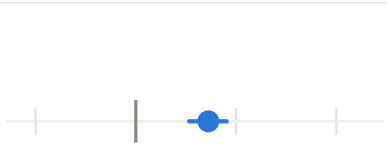
Harm to self (suicide)

General-population baseline: US suicide deaths: 13.7 per 100,000 per year (2024, age-adjusted; men 22.3, women 5.6; men ≥75: 40.1). Past-year attempts: ~0.8% of adults.

Risk factors, highest to lowest (54)

#	Factor	Estimate	0.1× 1× 10× 100× log scale · grey line = general population (1×)	Comparator	Outcome	Source	Comparison group & notes
1	**Acute alcohol use (now or in past 24 h): Heavy acute use / intoxication** Acute ✓✓ both sources hours-long window ⚑ red flag	**37.2** OR · 17.4–79.5		General population	suicide attempt	Borges 2017, Psychol Med [6]	no acute use · Risk window is hours, so excluded from the combined estimate. Four studies.
2	**4 or more adverse childhood experiences** Chronic	**30.1** OR · 14.7–61.7		General population	suicide attempt	Hughes 2017, Lancet Public Health [7]	adults with 0 ACEs · Cumulative-exposure contrast; overlaps with the abuse items below, so tick one or the other.
3	**Bipolar disorder** Chronic ≠ sources differ	**18.8** RR · 16–22.2		General population	suicide death	Webb 2014, J Clin Psychiatry [8]	matched population controls, Sweden · Sources differ: Swedish registry RR 18.8 (Webb 2014; consistent with SMR 17 in Chesney 2014) vs psychological-autopsy OR 4.6 (2.1–10.1; Favril 2022). Registry figure shown.

#	Factor	Estimate	0.1× 1× 10× 100× log scale · grey line = general population (1×)	Comparator	Outcome	Source	Comparison group & notes
4	**Suicidal ideation (C-SSRS severity, past month): Active with some intent, no plan (C-SSRS 4) / Active with specific plan and intent (C-SSRS 5)** Acute ✓✓ both sources derived ⚑ red flag	**18.3** RR · 5–67.3		General population	suicide death	Favril 2023, Lancet Public Health [9]	derived: 5.6 × 3.26; approximate CI · Mixes a death outcome (any ideation) with an attempt outcome (intent/plan increment). Posner 2011 did not separate level 4 from level 5, so both share this figure. · Components: Any ideation vs none RR 5.6 (3.1–10.1) [9]; × intent/ plan vs ideation without intent OR 3.26 (1.02–10.4) [1]
5	**Prior suicide attempt or self-harm** Chronic ✓✓ both sources	**16.3** OR · 7.5–35.5		General population	suicide death	Favril 2023, Lancet Public Health [9]	Strongest chronic predictor. Within clinical cohorts the incremental OR is much smaller (≈2.2; Franklin 2017).
6	**Opioid use disorder** Chronic	**13.5** SMR · no CI		General population	suicide death	Chesney 2014, World Psychiatry [10]	No pooled CI; intentional vs accidental overdose often misclassified.
7	**Depressive disorder** Chronic ✓✓ both sources	**11** OR · 7.3–16.5		General population	suicide death	Favril 2022, Evid Based Ment Health [11]	psychological-autopsy community controls · All mood disorders vs general population: RR 12.3 (Favril 2023).
8	**Major adverse life event, loss or humiliation in past month** Acute	**10.4** OR · 7.1–15.3		General population	suicide death	Favril 2022, Evid Based Ment Health [11]	Steep decay: OR 2.9 at 3 months.
9	**Borderline personality disorder** Chronic ✓✓ both sources	**9** OR · 5.6–14.4		General population	suicide death	Favril 2022, Evid Based Ment Health [11]	
10	**Released from prison in past year** Acute	**8.3** SMR · 7.5–9.3		General population	suicide death	Pratt 2006, Lancet [12]	age-matched male population, England & Wales · Women: SMR 35.8. 21% of post-release suicides occur in the first 28 days.
11	**Schizophrenia-spectrum disorder** Chronic ✓✓ both sources	**7.8** OR · 4.5–13.5		General population	suicide death	Favril 2022, Evid Based Ment Health [11]	psychological-autopsy community controls · Umbrella-review RR 13.2 (8.6–20.3; Favril 2023). Highest early in illness.
12	**Firearm access: Patient owns a handgun (women)** Lethal means ✓✓ both sources	**7.16** HR · 6.22–8.24		General population	suicide death	Studdert 2020, N Engl J Med [13]	female non-owners, California cohort · Firearm-suicide HR 35.15.

#	Factor	Estimate	0.1× 1× 10× 100× log scale · grey line = general population (1×)	Comparator	Outcome	Source	Comparison group & notes
13	**Acute alcohol use (now or in past 24 h): Any acute alcohol use** Acute ✓✓ both sources hours-long window	**6.97** OR · 4.77–10.2		General population	suicide attempt	Borges 2017, Psychol Med [6]	no acute use; case-control & case-crossover · Risk window is hours, so excluded from the combined (annual-scale) estimate. Low-dose OR 2.71.
14	**Anorexia nervosa** Chronic ✓✓ both sources	**6.9** RR · 4.1–11.5		General population	suicide death	Favril 2023, Lancet Public Health [9]	
15	**ADHD** Chronic	**6.7** OR · 3.2–13.8		General population	suicide death	Favril 2023, Lancet Public Health [9]	Wide prediction interval; largely mediated by impulsivity and comorbidity.
16	**Suicidal ideation (C-SSRS severity, past month): Wish to be dead (passive) / Active thoughts, no method / Active with method, no intent** Acute ✓✓ both sources	**5.6** RR · 3.1–10.1		General population	suicide death	Favril 2023, Lancet Public Health [9]	
17	**PTSD** Chronic ≠ sources differ	**5.3** OR · 3.4–8.1		General population	suicide death	Gradus 2010, Am J Epidemiol [14]	Danish population controls · Sources differ: Danish population case-control OR 5.3, adjusted for other psychiatric diagnoses (Gradus 2010) vs mixed-sample meta-analysis RR 2.09 (1.11–3.94; Akbar 2023). Population figure shown.
18	**Recent relationship conflict or breakup** Acute ✓✓ both sources	**5** OR · 3.3–7.6		General population	suicide death	Favril 2022, Evid Based Ment Health [11]	
19	**Legal problems / pending charges** Acute ✓✓ both sources	**4.8** OR · 2.4–9.4		General population	suicide death	Favril 2022, Evid Based Ment Health [11]	
20	**Criminal offending history** Chronic ✓✓ both sources	**4.5** OR · 2.1–9.7		General population	suicide death	Favril 2023, Lancet Public Health [9]	
21	**Substance use disorder (any)** Chronic ✓✓ both sources	**4.4** RR · 2.9–6.8		General population	suicide death	Favril 2023, Lancet Public Health [9]	
22	**History of non-suicidal self-injury** Chronic	**4.15** OR · 2.89–6.92		Clinical comparator	suicide attempt	Franklin 2017, Psychol Bull [15]	longitudinal cohorts without NSSI (mixed clinical/community)
23	**Anxiety disorder** Chronic	**4.1** RR · 2.4–6.9		General population	suicide death	Favril 2023, Lancet Public Health [9]	

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24	**Stopped or nonadherent with treatment** Acute	**3.75** OR · 2.2–6.37		Clinical comparator	suicide death	Hawton 2005, Br J Psychiatry [3]	adherent patients with schizophrenia · Abrupt lithium or clozapine discontinuation is especially risky.
25	**Prior psychiatric hospitalization** Chronic	**3.57** OR · 2.81–4.53		Clinical comparator	suicide death	Franklin 2017, Psychol Bull [15]	longitudinal cohorts without prior hospitalization · Any psychiatric treatment history vs community controls: OR 10.5 (Favril 2022).
26	**Transgender identity** Chronic	**3.5** IRR · 2–6.3		General population	suicide death	Erlangsen 2023, JAMA [16]	non-transgender Danish population · Suicide attempts IRR 7.7.
27	**Antisocial personality disorder** Chronic ✓✓ both sources	**3.4** OR · 2–6.1		General population	suicide death	Favril 2022, Evid Based Ment Health [11]	
28	**Foster or state care in childhood** Chronic	**3.4** RR · 2.4–4.7		General population	suicide death	Favril 2023, Lancet Public Health [9]	
29	**Firearm access: Patient owns a handgun (men)** Lethal means ✓✓ both sources	**3.34** HR · 3.13–3.56		General population	suicide death	Studdert 2020, N Engl J Med [13]	male non-owners, California cohort · Firearm-suicide HR 7.82; no rise in non-firearm suicide.
30	**Firearm access: Firearm in the household** Lethal means ✓✓ both sources	**3.24** OR · 2.41–4.4		General population	suicide death	Anglemyer 2014, Ann Intern Med [17]	household members without firearm access
31	**Alcohol use disorder** Chronic	**3.2** OR · 2.3–4.4		General population	suicide death	Favril 2022, Evid Based Ment Health [11]	Overlaps with "any SUD": tick one.
32	**Childhood sexual abuse** Chronic	**3.17** OR · 2.76–3.64		General population	suicide attempt	Angelakis 2019, Psychol Med [18]	Mostly retrospective.
33	**Parent or first-degree relative died by suicide** Chronic ✓✓ both sources	**3** RR · 2.5–3.5		General population	suicide death	Favril 2023, Lancet Public Health [9]	Any family history of suicide: OR 3.7 (Favril 2022).
34	**Epilepsy** Chronic ✓✓ both sources	**2.9** RR · 2.2–3.8		General population	suicide death	Favril 2023, Lancet Public Health [9]	
35	**Acute financial problems** Acute	**2.8** OR · 2–4		General population	suicide death	Favril 2022, Evid Based Ment Health [11]	
36	**Sex and age (demographic baseline): Male, age 75 or older** Chronic	**2.78** Rate ratio · no CI		General population	suicide death	CDC NCHS Data Brief 572 (2024 data) [19]	40.1 vs 14.4 per 100k overall (2024, crude) · Highest-rate US group; firearms predominate.
37	**Childhood physical abuse** Chronic	**2.52** OR · 2.09–3.04		General population	suicide attempt	Angelakis 2019, Psychol Med [18]	

#	Factor	Estimate	0.1× 1× 10× 100× log scale · grey line = general population (1×)	Comparator	Outcome	Source	Comparison group & notes
38	**Childhood emotional abuse** Chronic	**2.49** OR · 1.64–3.77		General population	suicide attempt	Angelakis 2019, Psychol Med [18]	
39	**Living alone / socially isolated** Chronic	**2.3** OR · 1.5–3.4		General population	suicide death	Favril 2022, Evid Based Ment Health [11]	Subjective social isolation OR 4.0.
40	**Lesbian, gay or bisexual identity** Chronic	**2.26** OR · 1.6–3.2		General population	suicide attempt	Miranda-Mendizábal 2017, Br J Psychiatry [20]	heterosexual peers (youth/young adult cohorts) · No death-rate figure: orientation not recorded on death certificates.
41	**Preparatory behavior (note, giving away possessions, rehearsal)** Acute ⚑ red flag	**2.15** HR · 1.27–3.62		Clinical comparator	suicide death	Saulnier 2025, JAMA Netw Open [21]	veterans assessed for suicide risk without this item · 30-day outcome, adjusted.
42	**Seeking access to lethal means** Lethal means ⚑ red flag	**2.04** HR · 1.11–3.75		Clinical comparator	suicide death	Saulnier 2025, JAMA Netw Open [21]	veterans assessed for suicide risk without this item
43	**Concussion / traumatic brain injury** Chronic ✓✓ both sources	**2** RR · 1.5–2.8		General population	suicide death	Favril 2023, Lancet Public Health [9]	
44	**Hopelessness** Acute	**1.98** OR · 1.46–2.69		Clinical comparator	suicide death	Ribeiro 2018, Br J Psychiatry [22]	longitudinal cohorts without elevated hopelessness · Within schizophrenia: OR 4.64 (Cassidy 2018).
45	**COPD** Chronic ✓✓ both sources	**1.9** OR · 1.3–2.9		General population	suicide death	Favril 2023, Lancet Public Health [9]	
46	**Cancer** Chronic ✓✓ both sources	**1.9** SMR · 1.6–2.2		General population	suicide death	Favril 2023, Lancet Public Health [9]	Highest in the first 6–12 months after diagnosis.
47	**Unemployment** Chronic	**1.9** OR · 1.4–2.5		General population	suicide death	Favril 2023, Lancet Public Health [9]	Psychological-autopsy OR 3.8 (Favril 2022).
48	**Agitation, severe anxiety or panic** Acute	**1.8** HR · 1.16–2.81		Clinical comparator	suicide death	Saulnier 2025, JAMA Netw Open [21]	veterans assessed for suicide risk without this item · Schizophrenia: agitation OR 2.61 (Hawton 2005). Consider akathisia.
49	**Insomnia / sleep disturbance** Acute ✓✓ both sources	**1.8** RR · 1.3–2.4		General population	suicide death	Favril 2023, Lancet Public Health [9]	

#	Factor	Estimate	0.1× 1× 10× 100× log scale · grey line = general population (1×)	Comparator	Outcome	Source	Comparison group & notes
50	**US military veteran** Chronic (≠ sources differ)	**1.72** Rate ratio · no CI		General population	suicide death	US VA 2024 Annual Report [23]	non-veteran US adults, age/sex adjusted · Sources differ: VA age- and sex-adjusted rate 71.8% higher than non-veteran adults (Gemini) vs indirectly standardized SMR 1.18 for 2023 (VA 2025 report; GPT). Older figure shown.
51	**Sex and age (demographic baseline): Male** Chronic (✓✓ both sources)	**1.63** Rate ratio · no CI		General population	suicide death	CDC NCHS Data Brief 572 (2024 data) [19]	22.3 vs 13.7 per 100k overall (2024, age-adjusted)
52	**Diabetes** Chronic (✓✓ both sources)	**1.6** RR · 1.3–1.9		General population	suicide death	Favril 2023, Lancet Public Health [9]	
53	**Chronic non-cancer pain** Chronic	**1.33** HR · 1.22–1.45		Selected subgroup	suicide death	Ilgen 2013, JAMA Psychiatry [24]	VHA patients without the pain condition · Back pain shown (99% CI); psychogenic pain HR 2.61.
54	**Recent bereavement** Acute (✓✓ both sources)	**1.2** OR · 0.50–3.1		General population	suicide death	Favril 2022, Evid Based Ment Health [11]	Imprecise; CI includes 1. Spousal loss in older men may matter more.

Protective factors, highest to lowest (10)

Values below 1 lower risk; the furthest from 1 (bottom of the table) are the most protective.

#	Factor	Estimate	0.1× 1× 10× 100× log scale · grey line = general population (1×)	Comparator	Outcome	Source	Comparison group & notes
1	**Taking clozapine (schizophrenia)** Protective (✓✓ both sources)	**0.76** HR · 0.58–0.97		Clinical comparator	suicide attempt	Meltzer 2003, Arch Gen Psychiatry (InterSePT) [25]	olanzapine, high-risk RCT patients · Finnish cohort: suicide death HR 0.34 vs perphenazine (Tiihonen 2009).
2	**On methadone/buprenorphine for opioid use disorder** Protective	**0.60** HR · 0.40–0.88		Clinical comparator	suicide attempt	Molero 2018, Am J Psychiatry [26]	same people off medication (within-person) · Methadone shown.
3	**Completed safety plan with follow-up contact** Protective (✓✓ both sources)	**0.56** OR · 0.33–0.95		Clinical comparator	suicide attempt	Stanley 2018, JAMA Psychiatry [27]	usual care, ED patients
4	**Married / stable partnership** Protective	**0.52** OR · 0.47–0.57		General population	suicide death	Kyung-Sook 2018, Soc Sci Med [28]	non-married adults (inverted from OR 1.92)
5	**Parent of children (responsibility for children)** Protective	**0.42** HR · 0.36–0.48		General population	suicide death	Dehara 2021, Acta Psychiatr Scand [29]	childless Swedish adults 25–44 · Psychological-autopsy data are weaker (childlessness OR 1.3, NS).

#	Factor	Estimate	0.1× 1× 10× 100× log scale · grey line = general population (1×)	Comparator	Outcome	Source	Comparison group & notes
6	**Sex and age (demographic baseline): Female** Chronic ✓✓ both sources	**0.41** Rate ratio · no CI		General population	suicide death	CDC NCHS Data Brief 572 (2024 data) [19]	5.6 vs 13.7 per 100k overall (2024) · Women attempt more often; men die more often.
7	**Firearms stored locked and unloaded** Protective ✓✓ both sources	**0.39** OR · 0.24–0.66		Selected subgroup	suicide death	Shenassa 2004, J Epidemiol Community Health [30]	unlocked storage; decedent controls · Firearm suicide; a 2025 reanalysis (Miller) found weaker adult evidence. Off-site storage is preferred in crisis.
8	**Strong reasons for living** Protective	**0.32** HR · 0.17–0.60		Selected subgroup	suicide attempt	Lizardi 2007, J Nerv Ment Dis [31]	depressed female inpatients with lower scores · Not significant in men (HR 0.83).
9	**Taking lithium (mood disorder)** Protective ✓✓ both sources	**0.30** OR · 0.09–1.02		Clinical comparator	suicide death	Riblet 2022, BJPsych Open [32]	placebo/usual care in RCTs · Older meta-analysis OR 0.13 (0.03–0.66; Cipriani 2013). Newer synthesis shown.
10	**Weekly religious service attendance** Protective ✓✓ both sources	**0.16** HR · 0.06–0.46		Selected subgroup	suicide death	VanderWeele 2016, JAMA Psychiatry [33]	never-attending US women nurses · Observational, low event count.

Factors without a usable figure (6)

No defensible general-population estimate exists, but several are among the strongest clinical warning signs.

- **Attempt, interrupted or aborted attempt in past 3 months** ⚑ red flag (Acute). Recency sharply amplifies the lifetime-attempt association; no single general-population figure. [1]
- **Discharged from psychiatric inpatient care in past 3 months** ⚑ red flag (Acute). Suicide rate ≈2,950/100k person-years in week 1, ≈2,060 in month 1 (Chung 2019) and ≈1,132 over 3 months (Chung 2017), vs 13.7 in the US population: roughly 80–200×. Pooled international rates, so not entered as an RR. [2]
- **Command hallucinations to self-harm** (Acute). No usable estimate: hallucinations in general showed an inverse association in schizophrenia (OR 0.50), likely confounded. Assess content, intent and compliance. [3]
- **Acquired a firearm in the past month** ⚑ red flag (Lethal means). Sources differ on the figure: Wintemute 1999 found a first-week firearm-suicide rate ~57× the general population; Gemini reported a Studdert 2020 first-month HR ≈100 that GPT could not verify. Not entered as a number. [4]
- **Supportive relationships** (Protective). No pooled general-population figure both sources agreed on. [5]
- **Engaged in ongoing treatment** (Protective). [5]

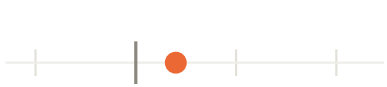
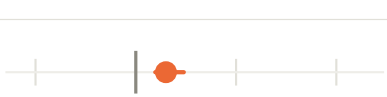
Harm to others (violence)

General-population baseline: US homicide: 5.9 deaths per 100,000 per year (2024; 6.2 age-adjusted). Past-year violent behavior: ~3.7% of adults (2.1% without mental disorder; ECA 1990).

Risk factors, highest to lowest (41)

#	Factor	Estimate	0.1× 1× 10× 100× log scale · grey line = general population (1×)	Comparator	Outcome	Source	Comparison group & notes
1	**Acute alcohol intoxication (past 24 h)** Acute · hours-long window · ⚑ red flag	**13.2** RR · 8.2–21.2		General population	violence	Haggård-Grann 2006, Addiction [35]	same offenders when not intoxicated (case-crossover) · Risk window is hours, so excluded from the combined estimate.
2	**Psychotic disorder (schizophrenia spectrum): Psychosis with comorbid substance use disorder** Chronic · ✓✓ both sources	**8.9** OR · 5.4–14.7		General population	violence	Fazel 2009, PLoS Med [36]	Joint exposure: similar to SUD alone. Do not also tick SUD.
3	**4 or more adverse childhood experiences** Chronic	**8.1** OR · 5.87–11.2		General population	violence	Hughes 2017, Lancet Public Health [7]	adults with 0 ACEs · Overlaps with the abuse items: tick one or the other.
4	**Antisocial personality disorder** Chronic · ✓✓ both sources	**7.6** OR · 5.1–11.5		General population	violence	Chow 2025, Br J Psychiatry [37]	
5	**Abusive partner has firearm access** Partner violence · ✓✓ both sources · ⚑ red flag	**7.59** OR · 3.85–15		Selected subgroup	partner homicide	Campbell 2003, Am J Public Health [38]	other abused women · Fully adjusted model: OR 5.38.
6	**Has strangled/choked partner before** Partner violence · ✓✓ both sources · ⚑ red flag	**7.48** OR · 4.53–12.3		Selected subgroup	partner homicide	Glass 2008, J Emerg Med [39]	other abused women
7	**Substance use disorder (alcohol or drugs)** Chronic	**7.4** OR · 4.3–12.7		General population	violence	Fazel 2018, Br J Psychiatry [40]	Largest population-attributable fraction for violence (~15%). In psychosis samples: OR 2.41 (Lagerberg 2025).
8	**PTSD** Chronic · ≠ sources differ	**6.4** HR · 5.7–7.2		General population	violence	Paulino 2023, Lancet Public Health [41]	Sources differ: Swedish population cohort HR 6.4 (sibling HR 3.2; Paulino 2023) vs veteran-only meta-analysis OR 1.59 (Taylor 2020). Population figure shown.
9	**History of violent victimization** Chronic · ✓✓ both sources	**5.81** OR · 3.45–9.78		Clinical comparator	violence	Lagerberg 2025, Br J Psychiatry [42]	other people with psychosis · Witt 2013: OR 6.1.

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10	**Abusive partner unemployed** Partner violence	**5.09** OR · 2.74–9.45		Selected subgroup	partner homicide	Campbell 2003, Am J Public Health [38]	partners employed full-time
11	**Abusive partner uses illicit drugs** Partner violence	**4.76** OR · 2.19–10.3		Selected subgroup	partner homicide	Campbell 2003, Am J Public Health [38]	other abused women
12	**Has threatened partner with a weapon** Partner violence	**4.08** OR · 1.91–8.72		Selected subgroup	partner homicide	Campbell 2003, Am J Public Health [38]	other abused women
13	**Persecutory delusions with anger** Acute	**3.68** OR · 2.44–5.55		Selected subgroup	violence	Keers 2014, Am J Psychiatry [43]	released prisoners without these delusions · Threat/control-override symptoms without anger: OR 1.2, NS (Witt 2013).
14	**Recent separation after living together** Partner violence ✓✓ both sources	**3.64** OR · 1.71–7.78		Selected subgroup	partner homicide	Campbell 2003, Am J Public Health [38]	abused women still cohabiting · Separation from a highly controlling partner: OR 8.98.
15	**Early-onset violence / youth antisocial behavior** Chronic	**3.6** OR · 3.4–3.7		General population	violence	Fazel 2018, Br J Psychiatry [40]	
16	**Violence in the past year** Acute ✓✓ both sources	**3.4** OR · 1.65–7		Clinical comparator	violence	Lagerberg 2025, Br J Psychiatry [42]	other people with psychosis
17	**Poor impulse control** Acute ✓✓ both sources	**3.3** OR · 1.5–7.2		Clinical comparator	violence	Witt 2013, PLoS ONE [44]	lower scores within psychosis samples
18	**Traumatic brain injury** Chronic ✓✓ both sources	**3.3** OR · 3.1–3.5		General population	violence	Fazel 2011, PLoS Med [45]	Sibling comparison OR 2.0; earlier meta-analysis OR 1.66.
19	**History of incarceration** Chronic	**3.04** OR · 2.08–4.44		Clinical comparator	violence	Lagerberg 2025, Br J Psychiatry [42]	other people with psychosis
20	**Depressive disorder** Chronic ✓✓ both sources	**3** OR · 2.8–3.3		General population	violence	Fazel 2015, Lancet Psychiatry [46]	matched population controls, Sweden · Sibling comparison OR 1.5.
21	**History of violence** Chronic ≠ sources differ	**2.91** OR · 2.06–4.1		Clinical comparator	violence	Lagerberg 2025, Br J Psychiatry [42]	other people with psychosis · Most consistent predictor across settings (HCR-20 H1). · Sources differ: longitudinal psychosis studies OR 2.91 (Lagerberg 2025) vs mixed-design OR 21.4 (5.2–86.6; Witt 2013). Conservative figure shown.
22	**Hostile behavior** Acute	**2.8** OR · 1.8–4.2		Clinical comparator	violence	Witt 2013, PLoS ONE [44]	non-hostile people with psychosis

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23	**Lack of insight into illness** Acute ✓✓ both sources	**2.7** OR · 1.4–5.2		Clinical comparator	violence	Witt 2013, PLoS ONE [44]	other people with psychosis
24	**Borderline personality disorder** Chronic ✓✓ both sources	**2.6** OR · 1.8–3.9		General population	violence	Chow 2025, Br J Psychiatry [37]	
25	**Has threatened to kill partner** Partner violence	**2.6** OR · 1.24–5.42		Selected subgroup	partner homicide	Campbell 2003, Am J Public Health [38]	other abused women
26	**Discharged from psychiatric inpatient care in past 10 weeks** Acute	**2.5** RR · no CI		General population	violence	Steadman 1998, Arch Gen Psychiatry [47]	neighborhood controls (11.5% vs 4.6%) · No difference from neighbors when substance abuse was absent (4.7% vs 3.3%).
27	**Patient owns a handgun and lives with others** Lethal means	**2.33** HR · 1.78–3.05		General population	homicide of household member	Studdert 2022, Ann Intern Med [48]	cohabitants of non-owners, California · At-home firearm homicide by an intimate partner: HR 7.16.
28	**Bipolar disorder** Chronic ✓✓ both sources	**2.3** OR · 2–2.6		General population	violence	Fazel 2010, Arch Gen Psychiatry [49]	matched population controls, Sweden · Driven by comorbid SUD (6.4 with vs 1.3 without).
29	**Childhood physical abuse** Chronic ✓✓ both sources	**2.2** OR · 1.5–3.1		Clinical comparator	violence	Witt 2013, PLoS ONE [44]	other people with psychosis
30	**Psychotic disorder (schizophrenia spectrum): Psychosis without substance use disorder** Chronic ✓✓ both sources	**2.1** OR · 1.7–2.7		General population	violence	Fazel 2009, PLoS Med [36]	
31	**Firearm in the household** Lethal means ✓✓ both sources	**2** OR · 1.56–3.02		General population	homicide (household member victimization)	Anglemyer 2014, Ann Intern Med [17]	Measures household members being killed, not the patient as perpetrator.
32	**Medication nonadherence** Acute ✓✓ both sources	**2** OR · 1–3.7		Clinical comparator	violence	Witt 2013, PLoS ONE [44]	adherent people with psychosis
33	**Age under 25** Chronic	**1.9** OR · 1.6–2.3		Selected subgroup	violence	Yukhnenko 2019, CNS Spectr [50]	older offenders (recidivism studies) · Violence peaks in late teens and early 20s.
34	**Childhood sexual abuse** Chronic	**1.9** OR · 1.5–2.4		Clinical comparator	violence	Witt 2013, PLoS ONE [44]	other people with psychosis
35	**Poor sleep / sleep loss** Acute	**1.83** OR · 1.47–2.28		Selected subgroup	violence	van Veen 2022, Sleep Med Rev [51]	adequate sleep, observational studies
36	**ADHD** Chronic	**1.8** OR · 1.6–1.9		General population	violence	Fazel 2018, Br J Psychiatry [40]	Largely mediated by conduct disorder and SUD.

#	Factor	Estimate	0.1× 1× 10× 100× log scale · grey line = general population (1×)	Comparator	Outcome	Source	Comparison group & notes
37	**Recent drug or alcohol misuse** Acute ≠ sources differ	**1.8** OR · 1.35–2.38		Clinical comparator	violence	Lagerberg 2025, Br J Psychiatry [42]	other people with psychosis · Sources differ: OR 1.80 (Lagerberg 2025) vs 2.9 (1.3–6.3; Witt 2013).
38	**Sex (demographic baseline): Male** Chronic	**1.8** Rate ratio · no CI		General population	homicide perpetration	BJS / FBI Supplementary Homicide Reports [52]	derived: 88% of offenders vs ~49% of population · Male:female offending ≈7:1. Within psychiatric inpatients the sex gap narrows.
39	**Agitation / excitement** Acute ✓✓ both sources	**1.6** OR · 1–2.6		Clinical comparator	violence	Witt 2013, PLoS ONE [44]	lower scores within psychosis samples
40	**Parental violent crime** Chronic	**1.37** OR · 1.15–1.63		Clinical comparator	violence	Lagerberg 2025, Br J Psychiatry [42]	other people with psychosis
41	**Command hallucinations to harm others** Acute ⚑ red flag	**1** OR · 0.50–2		Clinical comparator	violence	Witt 2013, PLoS ONE [44]	people with psychosis without commands · Unselected commands show no effect, but harm-specific commands roughly double recent violence (McNiel 2000).

Protective factors, highest to lowest (7)

Values below 1 lower risk; the furthest from 1 (bottom of the table) are the most protective.

#	Factor	Estimate	0.1× 1× 10× 100× log scale · grey line = general population (1×)	Comparator	Outcome	Source	Comparison group & notes
1	**Taking a mood stabilizer** Protective ✓✓ both sources	**0.76** HR · 0.62–0.93		Clinical comparator	violence	Fazel 2014, Lancet [53]	same patients off medication · Effect seen mainly in bipolar disorder.
2	**On buprenorphine/methadone for opioid use disorder** Protective	**0.65** HR · 0.50–0.84		Clinical comparator	violence	Molero 2018, Am J Psychiatry [26]	same people off medication (within-person) · Buprenorphine shown; methadone HR 0.84.
3	**Adherent to treatment** Protective	**0.59** OR · 0.33–1.06		Clinical comparator	violence	Lagerberg 2025, Br J Psychiatry [42]	nonadherent people with psychosis · CI crosses 1.
4	**Taking antipsychotic medication** Protective ✓✓ both sources	**0.55** HR · 0.47–0.64		Clinical comparator	violence	Fazel 2014, Lancet [53]	same patients off medication (within-person) · Long-acting injectables: HR 0.60.

#	Factor	Estimate	0.1× 1× 10× 100× log scale · grey line = general population (1×)	Comparator	Outcome	Source	Comparison group & notes
5	**Taking clozapine** Protective ≠ sources differ	**0.53** HR · 0.16–1.74		Clinical comparator	violence	Fazel 2014, Lancet [53]	same patients off clozapine · Sources differ: within-person HR 0.53, imprecise (Fazel 2014) vs mirror-image RR 0.13 (0.05–0.34; Bhavsar 2020). Conservative figure shown.
6	**Active prosocial family/friend support** Protective	**0.37** OR · 0.20–0.66		Selected subgroup	violence	Ullrich & Coid 2011, J Consult Clin Psychol [54]	released male prisoners without this support
7	**Sex (demographic baseline): Female** Chronic	**0.24** Rate ratio · no CI		General population	homicide perpetration	BJS / FBI Supplementary Homicide Reports [52]	derived: 12% of offenders vs ~51% of population

Factors without a usable figure (5)

No defensible general-population estimate exists, but several are among the strongest clinical warning signs.

- **Psychopathic traits** (Chronic). PCL-R studies report effect sizes (d ≈ 0.6) in forensic samples, not general-population ORs. [34]
- **Threats toward an identifiable person** ⚑ red flag (Acute). Assess target, specificity, intent, means and escalation. Among people convicted of threats to kill, 3.1% later committed homicide within 10 years. [34]
- **Homicidal ideation with plan or intent** ⚑ red flag (Acute). [34]
- **Recently acquired or sought a weapon** ⚑ red flag (Lethal means). Context, target and motive matter. [34]
- **Firearms removed or secured off-site** (Protective). [34]

References

1. Posner K, Brown GK, Stanley B, et al. The Columbia–Suicide Severity Rating Scale: initial validity and internal consistency findings from three multisite studies with adolescents and adults. Am J Psychiatry. 2011;168(12):1266–1277. PMID 22193671 https://pubmed.ncbi.nlm.nih.gov/22193671/
2. Chung D, Hadzi-Pavlovic D, Wang M, Swaraj S, Olfson M, Large M. Meta-analysis of suicide rates in the first week and the first month after psychiatric hospitalisation. BMJ Open. 2019;9(3):e023883. PMID 30904843 https://pubmed.ncbi.nlm.nih.gov/30904843/
3. Hawton K, Sutton L, Haw C, Sinclair J, Deeks JJ. Schizophrenia and suicide: systematic review of risk factors. Br J Psychiatry. 2005;187:9–20. PMID 15994566 https://pubmed.ncbi.nlm.nih.gov/15994566/
4. Wintemute GJ, Parham CA, Beaumont JJ, Wright M, Drake C. Mortality among recent purchasers of handguns. N Engl J Med. 1999;341(21):1583–1589. PMID 10564688 https://pubmed.ncbi.nlm.nih.gov/10564688/
5. Substance Abuse and Mental Health Services Administration. Suicide Assessment Five-Step Evaluation and Triage (SAFE-T). PEP24-01-036. 2024. https://library.samhsa.gov/sites/default/files/safet-flyer-pep24-01-036.pdf
6. Borges G, Bagge CL, Cherpitel CJ, Conner KR, Orozco R, Rossow I. A meta-analysis of acute use of alcohol and the risk of suicide attempt. Psychol Med. 2017;47(5):949–957. PMID 27928972 https://pmc.ncbi.nlm.nih.gov/articles/PMC5340592/
7. Hughes K, Bellis MA, Hardcastle KA, et al. The effect of multiple adverse childhood experiences on health: a systematic review and meta-analysis. Lancet Public Health. 2017;2(8):e356–e366. PMID 29253477 https://pubmed.ncbi.nlm.nih.gov/29253477/
8. Webb RT, Lichtenstein P, Larsson H, Geddes JR, Fazel S. Suicide, hospital-presenting suicide attempts, and criminality in bipolar disorder: examination of risk for multiple adverse outcomes. J Clin Psychiatry. 2014;75(8):e809–e816. doi:10.4088/JCP.13m08899 https://pmc.ncbi.nlm.nih.gov/articles/PMC4226039/
9. Favril L, Yu R, Geddes JR, Fazel S. Individual-level risk factors for suicide mortality in the general population: an umbrella review. Lancet Public Health. 2023;8(11):e868–e877. doi:10.1016/S2468-2667(23)00207-4 https://pmc.ncbi.nlm.nih.gov/articles/PMC10932753/
10. Chesney E, Goodwin GM, Fazel S. Risks of all-cause and suicide mortality in mental disorders: a meta-review. World Psychiatry. 2014;13(2):153–160. PMID 24890068 https://pubmed.ncbi.nlm.nih.gov/24890068/
11. Favril L, Yu R, Uyar A, Sharpe M, Fazel S. Risk factors for suicide in adults: systematic review and meta-analysis of psychological autopsy studies. Evid Based Ment Health. 2022;25(4):148–155. doi:10.1136/ebmental-2022-300549 https://doi.org/10.1136/ebmental-2022-300549
12. Pratt D, Piper M, Appleby L, Webb R, Shaw J. Suicide in recently released prisoners: a population-based cohort study. Lancet. 2006;368(9530):119–123. PMID 16829295 https://pubmed.ncbi.nlm.nih.gov/16829295/
13. Studdert DM, Zhang Y, Swanson SA, et al. Handgun ownership and suicide in California. N Engl J Med. 2020;382(23):2220–2229. PMID 32492303 https://pubmed.ncbi.nlm.nih.gov/32492303/
14. Gradus JL, Qin P, Lincoln AK, et al. Posttraumatic stress disorder and completed suicide. Am J Epidemiol. 2010;171(6):721–727. PMID 20164191 https://pubmed.ncbi.nlm.nih.gov/20164191/
15. Franklin JC, Ribeiro JD, Fox KR, et al. Risk factors for suicidal thoughts and behaviors: a meta-analysis of 50 years of research. Psychol Bull. 2017;143(2):187–232. PMID 27841450 https://pubmed.ncbi.nlm.nih.gov/27841450/
16. Erlangsen A, Jacobsen AL, Ranning A, Delamare AL, Nordentoft M, Frisch M. Transgender identity and suicide attempts and mortality in Denmark. JAMA. 2023;329(24):2145–2153. PMID 37367977 https://pubmed.ncbi.nlm.nih.gov/37367977/
17. Anglemyer A, Horvath T, Rutherford G. The accessibility of firearms and risk for suicide and homicide victimization among household members: a systematic review and meta-analysis. Ann Intern Med. 2014;160(2):101–110. doi:10.7326/M13-1301 https://doi.org/10.7326/M13-1301
18. Angelakis I, Gillespie EL, Panagioti M. Childhood maltreatment and adult suicidality: a comprehensive systematic review with meta-analysis. Psychol Med. 2019;49(7):1057–1078. PMID 30608046 https://pubmed.ncbi.nlm.nih.gov/30608046/
19. Garnett MF, Zehner AE. Suicide mortality in the United States, 2004–2024. NCHS Data Brief No. 572. National Center for Health Statistics; 2026. https://www.cdc.gov/nchs/products/databriefs/db572.htm
20. Miranda-Mendizábal A, Castellví P, Parés-Badell O, et al. Sexual orientation and suicidal behaviour in adolescents and young adults: systematic review and meta-analysis. Br J Psychiatry. 2017;211(2):77–87. PMID 28254960 https://pubmed.ncbi.nlm.nih.gov/28254960/
21. Saulnier KG, et al. Suicide risk factors and warning signs documented in the Comprehensive Suicide Risk Evaluation and subsequent suicide death among veterans. JAMA Netw Open. 2025. PMID 39998832 https://pubmed.ncbi.nlm.nih.gov/39998832/
22. Ribeiro JD, Huang X, Fox KR, Franklin JC. Depression and hopelessness as risk factors for suicide ideation, attempts and death: meta-analysis of longitudinal studies. Br J Psychiatry. 2018;212(5):279–286. PMID 29587888 https://pubmed.ncbi.nlm.nih.gov/29587888/
23. US Department of Veterans Affairs. National Veteran Suicide Prevention Annual Report (age- and sex-adjusted comparison with non-veteran US adults). https://www.mentalhealth.va.gov/suicide_prevention/data.asp
24. Ilgen MA, Kleinberg F, Ignacio RV, et al. Noncancer pain conditions and risk of suicide. JAMA Psychiatry. 2013;70(7):692–697. PMID 23699975 https://pubmed.ncbi.nlm.nih.gov/23699975/
25. Meltzer HY, Alphs L, Green AI, et al. Clozapine treatment for suicidality in schizophrenia: International Suicide Prevention Trial (InterSePT). Arch Gen Psychiatry. 2003;60(1):82–91. PMID 12511175 https://pubmed.ncbi.nlm.nih.gov/12511175/
26. Molero Y, Zetterqvist J, Binswanger IA, et al. Medications for alcohol and opioid use disorders and risk of suicidal behavior, accidental overdoses, and crime. Am J Psychiatry. 2018;175(10):970–978. PMID 30068260 https://pubmed.ncbi.nlm.nih.gov/30068260/
27. Stanley B, Brown GK, Brenner LA, et al. Comparison of the Safety Planning Intervention with follow-up vs usual care of suicidal patients treated in the emergency department. JAMA Psychiatry. 2018;75(9):894–900. PMID 29998307 https://pubmed.ncbi.nlm.nih.gov/29998307/
28. Kyung-Sook W, SangSoo S, Sangjin S, Young-Jeon S. Marital status integration and suicide: a meta-analysis and meta-regression. Soc Sci Med. 2018;197:116–126. PMID 29227910 https://pubmed.ncbi.nlm.nih.gov/29227910/

29. Dehara M, Wells MB, Sjöqvist H, et al. Parenthood is associated with lower suicide risk: a register-based cohort study of 1.5 million Swedes. Acta Psychiatr Scand. 2021;143(3):206–215. PMID 33067802 https://pubmed.ncbi.nlm.nih.gov/33067802/
30. Shenassa ED, Rogers ML, Spalding KL, Roberts MB. Safer storage of firearms at home and risk of suicide: a study of protective factors in a nationally representative sample. J Epidemiol Community Health. 2004;58(10):841–848. PMID 15365110 https://pubmed.ncbi.nlm.nih.gov/15365110/
31. Lizardi D, Currier D, Galfalvy H, et al. Perceived reasons for living at index hospitalization and future suicide attempt. J Nerv Ment Dis. 2007;195(5):451–455. PMID 17502812 https://pubmed.ncbi.nlm.nih.gov/17502812/
32. Riblet NB, Shiner B, Young-Xu Y, Watts BV. Lithium in the prevention of suicide in adults: systematic review and meta-analysis of clinical trials. BJPsych Open. 2022;8(6):e199. PMID 36384820 https://pubmed.ncbi.nlm.nih.gov/36384820/
33. VanderWeele TJ, Li S, Tsai AC, Kawachi I. Association between religious service attendance and lower suicide rates among US women. JAMA Psychiatry. 2016;73(8):845–851. PMID 27367927 https://pubmed.ncbi.nlm.nih.gov/27367927/
34. Douglas KS, Hart SD, Webster CD, Belfrage H. HCR-20 V3: Assessing Risk for Violence. Burnaby, BC: Mental Health, Law, and Policy Institute, Simon Fraser University; 2013. https://www.sfu.ca/psychology/research/drl/hcr-20--version-3-/
35. Haggård-Grann U, Hallqvist J, Långström N, Möller J. The role of alcohol and drugs in triggering criminal violence: a case-crossover study. Addiction. 2006;101(1):100–108. PMID 16393196 https://pubmed.ncbi.nlm.nih.gov/16393196/
36. Fazel S, Gulati G, Linsell L, Geddes JR, Grann M. Schizophrenia and violence: systematic review and meta-analysis. PLoS Med. 2009;6(8):e1000120. PMID 19668362 https://pubmed.ncbi.nlm.nih.gov/19668362/
37. Chow RTS, Yu R, Geddes JR, Fazel S. Personality disorders, violence and antisocial behaviour: updated systematic review and meta-regression analysis. Br J Psychiatry. 2025. doi:10.1192/bjp.2024.226 https://pmc.ncbi.nlm.nih.gov/articles/PMC12278050/
38. Campbell JC, Webster D, Koziol-McLain J, et al. Risk factors for femicide in abusive relationships: results from a multisite case control study. Am J Public Health. 2003;93(7):1089–1097. PMID 12835191 https://pmc.ncbi.nlm.nih.gov/articles/PMC1447915/
39. Glass N, Laughon K, Campbell J, et al. Non-fatal strangulation is an important risk factor for homicide of women. J Emerg Med. 2008;35(3):329–335. PMID 17961956 https://pubmed.ncbi.nlm.nih.gov/17961956/
40. Fazel S, Smith EN, Chang Z, Geddes JR. Risk factors for interpersonal violence: an umbrella review of meta-analyses. Br J Psychiatry. 2018;213(4):609–614. PMID 30058516 https://pubmed.ncbi.nlm.nih.gov/30058516/
41. Paulino S, et al. Post-traumatic stress disorder and violent crime: a Swedish population-based cohort and sibling-comparison study. Lancet Public Health. 2023. PMID 37244673 https://pubmed.ncbi.nlm.nih.gov/37244673/
42. Lagerberg T, et al. Risk factors for violence in psychosis: updated systematic review and meta-analysis of longitudinal studies. Br J Psychiatry. 2025. doi:10.1192/bjp.2024.120 https://pmc.ncbi.nlm.nih.gov/articles/PMC7617503/
43. Keers R, Ullrich S, DeStavola BL, Coid JW. Association of violence with emergence of persecutory delusions in untreated schizophrenia. Am J Psychiatry. 2014;171(3):332–339. PMID 24220644 https://pubmed.ncbi.nlm.nih.gov/24220644/
44. Witt K, van Dorn R, Fazel S. Risk factors for violence in psychosis: systematic review and meta-regression analysis of 110 studies. PLoS ONE. 2013;8(2):e55942. doi:10.1371/journal.pone.0055942 https://journals.plos.org/plosone/article?id=10.1371/journal.pone.0055942
45. Fazel S, Lichtenstein P, Grann M, Långström N. Risk of violent crime in individuals with epilepsy and traumatic brain injury: a 35-year Swedish population study. PLoS Med. 2011;8(12):e1001150. PMID 22215988 https://pubmed.ncbi.nlm.nih.gov/22215988/
46. Fazel S, Wolf A, Chang Z, Larsson H, Goodwin GM, Lichtenstein P. Depression and violence: a Swedish population study. Lancet Psychiatry. 2015;2(3):224–232. doi:10.1016/S2215-0366(14)00128-X https://pmc.ncbi.nlm.nih.gov/articles/PMC4520382/
47. Steadman HJ, Mulvey EP, Monahan J, et al. Violence by people discharged from acute psychiatric inpatient facilities and by others in the same neighborhoods. Arch Gen Psychiatry. 1998;55(5):393–401. PMID 9596041 https://pubmed.ncbi.nlm.nih.gov/9596041/
48. Studdert DM, Zhang Y, Holsinger EE, et al. Homicide deaths among adult cohabitants of handgun owners in California, 2004 to 2016: a cohort study. Ann Intern Med. 2022;175(6):804–811. PMID 35377715 https://pubmed.ncbi.nlm.nih.gov/35377715/
49. Fazel S, Lichtenstein P, Grann M, Goodwin GM, Långström N. Bipolar disorder and violent crime: new evidence from population-based longitudinal studies and systematic review. Arch Gen Psychiatry. 2010;67(9):931–938. PMID 20819987 https://pubmed.ncbi.nlm.nih.gov/20819987/
50. Yukhnenko D, Sridhar S, Fazel S. A systematic review of criminal recidivism rates worldwide: 3-year update. Wellcome Open Res / CNS Spectr. 2019. PMID 31185959 https://pubmed.ncbi.nlm.nih.gov/31185959/
51. van Veen MM, Lancel M, Şener O, Verkes RJ, Bouman EJ, Rutters F. Observational and experimental studies on sleep duration and aggression: a systematic review and meta-analysis. Sleep Med Rev. 2022;64:101661. PMID 35810548 https://pubmed.ncbi.nlm.nih.gov/35810548/
52. Cooper A, Smith EL. Homicide Trends in the United States, 1980–2008. Bureau of Justice Statistics; 2011; and FBI Uniform Crime Reporting Supplementary Homicide Reports (offender sex where known). https://bjs.ojp.gov/library/publications/homicide-trends-united-states-1980-2008
53. Fazel S, Zetterqvist J, Larsson H, Långström N, Lichtenstein P. Antipsychotics, mood stabilisers, and risk of violent crime. Lancet. 2014;384(9949):1206–1214. PMID 24816046 https://pubmed.ncbi.nlm.nih.gov/24816046/
54. Ullrich S, Coid J. Protective factors for violence among released prisoners: effects over time and interactions with static risk. J Consult Clin Psychol. 2011;79(3):381–390. PMID 21500889 https://pubmed.ncbi.nlm.nih.gov/21500889/

For clinical teaching only. If someone is in immediate danger call 911; for a suicidal crisis call or text 988 (US).